

Ireache Lastmile REACH 360o Community Health System strengthening framework
transforms health care delivery to reach everyone. Lastmile

Sierra Leone · Eastern Province · 2026

Building the health system Sierra Leone's communities have always deserved.

iREACHE LASTMILE connects three systems that have never governed community health together — traditional authority, government, and community intelligence — through 6,800 ProcCHWs reaching every last-mile household every month.

Our Model

Read Our Story

Our Impact

6,800

ProcCHWs across all 16 districts by 2031
All districts

4.5M

People in underserved and zero-dose communities By 2031

354

Maternal deaths per 100,000 — the number we are here to change
Target < 300

\$1

The cost to treat the infection that killed Mariama — 14 hours old
Why we exist

6,800
ProcCHWs by 2031

16
Districts
4.5M
People reached
354→300
MMR target
\$1
Cost of Mariama's cu

WHO WE ARE

"We are not the Sierra Leonean version of a global model. We are the first nationally-rooted community health systems institution Sierra Leone has produced — designed, from its founding moment, to build a community health system that belongs permanently to this country and to the people who live here."

We are Community Health System Strengthening organization working at the lastmile working to make primary health care reliable and accessible for the communities furthest from it.

iREACHE LASTMILE was founded on a different diagnosis — and a different solution.

We are not a community health delivery organization. We are a community health systems connection organization. Our model — LASTMILE CARE — puts Professional Connected Community Health Workers, ProcCHWs, at the doorstep of every household in our programme communities every single month. But a ProcCHW in our model is not a service delivery agent. They are a systems connector — the human infrastructures through which the Traditional Authority System, the Government Health System, and the Community Epidemiological System exchange intelligence, align authority, and govern community health together.

Our VISION

A Sierra Leone where every person — in every last-mile community, regardless of distance or poverty — receives proactive, dignified community health care from a professional workforce their own community governs and their own government sustains.

Our MISSION

iREACHE LASTMILE saves lives at the last mile by deploying Professional Connected Community Health Workers who bring proactive, AI-supported care to every doorstep — bridging traditional authority, government, and community into a health system that is community-governed and built to last.

Eight organizational values with icons and plain-language descriptions

Organizational Values

Value	What It Means in Practice at iREACHE LASTMILE
EQUITY	Equity is the spine of the REACH 360° Framework — not a cross-cutting theme or a compliance requirement. Every resource allocation, system design, AI tool deployment, and advocacy effort is directed first toward the communities most excluded from health outcomes. The communities with the highest burden receive the most intensive support. We measure success by narrowing the gap between the best-served and worst-served communities — not by improving programme averages while inequalities widen.
EXCELLENCE	We apply the highest technical and operational standards — modelled on Living Goods' supervision rigour, VillageReach's supply chain discipline, Muso's proactive care commitment, and WHO's Responsible AI Ethics principles. We do not deploy tools, protocols, or systems that have not been validated in Sierra Leone's specific context. We publish our methods and our results — including our failures — so that excellence is verifiable rather than claimed.
ACCOUNTABILITY	We are accountable first to communities, then to government, then to donors — in that order. Community Health Committees govern programme decisions. DHMTs co-lead all supervision. Toll-free accountability hotlines give every community member a direct line to programme leadership. Annual Open House meetings make programme results publicly available and publicly contestable. We do not manage accountability. We build it into programme architecture.
CO-OWNERSHIP	'Doing WITH, not doing FOR' is a programme design specification, not a communications aspiration. Communities select their ProcCHWs. Traditional Authority Health Governance Councils hold formal programme accountability. Community Health Committees make real governance decisions with real consequences. Co-ownership is the mechanism through which BRIDGE 360° LASTMILE CARE outlasts iREACHE LASTMILE's direct involvement. It is what transforms a programme into a system.
GOVERNMENT PRIMACY	Every system iREACHE LASTMILE builds is designed from its first day to be owned, financed, and operated by the Government of Sierra Leone. We build no parallel infrastructure. We create no data systems that the government cannot access independently. We design no digital health tools that require iREACHE LASTMILE's technical team to interpret. We

Value	What It Means in Practice at iREACHE LASTMILE
	measure our success by how little Sierra Leone needs iREACHE LASTMILE at the end of the programme — not by how much.
EVIDENCE	Every programme decision is grounded in data. The REACH 360° Crosswalk documents the evidence base for every framework element with the precision of a scientific citation — naming the specific international inspiration, the specific Sierra Leone evidence source, and the specific original contribution. We treat our implementation experience as research data and publish it through peer-reviewed journals, open-access Learning Briefs, and the annual REACH 360° Crosswalk update.

TAGLINE	<i>"Empowerment means doing WITH, not doing FOR."</i>
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OUR FOUNDING STORY

Origin Moment: Two encounters, one decision, one name written on a piece of paper

The First Encounter

In June 2018, Augustine Alie was working as a community development officer in Kenema district.

His work took him across the district regularly — to chieftaincy meetings, to community health consultations, to the planning meetings where DHMT staff and NGO representatives sat together and discussed programme targets.

One morning he drove three hours on a dirt road to attend a community health planning meeting in a remote chieftaincy. He arrived to find forty community members waiting. They had been waiting since eight in the morning. It was now eleven.

The NGO representative had not come. The vehicle had broken down somewhere between Kenema town and the chieftaincy. Nobody had sent word.

Augustine ran the meeting himself. He had not planned to. He did not have the programme documents or the agenda. He sat down with the forty community members and asked them the only question he could think of.

Tell me what health care looks like in your community right now. Not what you want. What is actually happening?

They talked for three hours.

What Augustine heard that morning — the specific texture of community health failure in Eastern Sierra Leone, described not through data but through lived experience — stayed with him for years. A grandmother describing how her granddaughter had died of a fever because the community health worker's medicine kit had been empty for six weeks. A young man describing the chief's frustration at being asked to mobilize community members for a programme that had collapsed three times in twelve years. A pregnant woman sitting quietly in the back of the room who said only one thing when Augustine looked at her directly.

I am afraid of this pregnancy. Not because I am sick, because I don't know if anyone will be there when it matters.

Augustine drove back to Kenema that afternoon with forty testimonies in his notebook and one question he could not answer.

Why does Sierra Leone keep building the same programmes and getting the same results?

The Second Encounter

Eight months later, Augustine was visiting his grandmother in her village in the Niawa chieftaincy of Kenema district.

His grandmother was seventy-four years old. She had lived through the civil war, through the Ebola outbreak, through more health programmes than she could count. She was sharp and direct and completely unimpressed by official explanations of anything.

On his second evening there, Augustine told her about the question he had been carrying since the community meeting eight months earlier.

She listened. She made tea. She was quiet for a long time.

Then she told him something he had not known.

She told him that during the Ebola outbreak in 2014, when the formal health response was slow to reach their chieftaincy, the community had organized itself. The paramount chief had convened an emergency council. Women's group leaders had gone house to house asking specific questions about symptoms. Section chiefs had set up alert chains. Trusted community members had become the conduits through which sick households communicated with the response teams — because families trusted their neighbors in a way they did not yet trust the government teams who were arriving in unfamiliar protective equipment.

She described this network in specific detail. Who had done what? How the alerts had moved. Which community decisions had contained transmission in specific neighborhoods?

We did these ourselves, she said, before anyone came to help us. We already knew how to protect each other. Nobody ever asked us about it afterward. Nobody wrote it down. Nobody used it for anything. The next programme that came just started training people as if we knew nothing.

She looked at Augustine steadily.

The knowledge is still here, she said, in this community. In every community that survived. Nobody has ever connected it to anything.

Augustine sat with his grandmother until late that night.

When he drove back to Kenema the following morning, he had the answer to the question he had been carrying for eight months.

Sierra Leone was not getting different results because it needed better programmes.

It was getting the same results because it was ignoring three systems that were already present in every community — the traditional authority that held social legitimacy, the government health architecture that held clinical authority, and the community's own epidemiological intelligence that had been built through survival and never activated.

He pulled over on the road between Niawa chieftaincy and Kenema town.

He took out his notebook. He wrote four words at the top of a blank page.

What if we connected them?

Below those four words, he began writing what would eventually become the REACH 360° Community Health Systems Strengthening Framework.

The Name

A week later, Augustine was in a small meeting room in Kenema with four colleagues — a public health physician, a community development specialist, a former DHMT officer, and a nurse who had worked in rural health posts for fifteen years.

He walked them through what he had been thinking.

They talked for two days.

On the second afternoon, they needed to name what they were building.

The nurse — a woman named Mariama Koroma, no relation to Augustine — wrote one sentence on the whiteboard.

We are building something for the people the system never reaches, the last mile, the forgotten mile.

Augustine looked at the sentence for a long time.

Then he wrote five words underneath it.

Innovations for Rural Empowerment — iREACHE.

And below that, three more words.

Last mile. Always.

The organization was named in that room in Kenema on a Tuesday afternoon in March 2019.

Its headquarters have been in Kenema ever since.

Not in Freetown. Not in a district capital closer to the international airport or the donor offices or the government ministries.

In Kenema, where the grandmother's knowledge lives, where the community meeting happened with forty people and no NGO representative. Where the question was first asked and where the answer is being built.

What We Are Building

iREACHE LASTMILE exists because two encounters — a community meeting where forty people described the texture of health failure in the most honest language Augustine had ever heard, and an evening with an elderly woman who described a community surveillance network that had saved lives and been forgotten — revealed something that ten years of programme evaluations and district health reports had not.

Sierra Leone does not need another community health programme.

Sierra Leone needs a community health system.

One that connects what already exists — the chief's social authority, the government's clinical capacity, the community's epidemiological intelligence — into a single governing architecture that is community-owned, government-integrated, and domestically financed.

One that, when asked Chief Fofanah's question — *when you go, what belongs to us* — can answer clearly.

Everything.

The governance belongs to the community. The data belongs to the government. The knowledge belongs to the people who built it by surviving Ebola. The financing belongs to Sierra Leone's own budget.

The only thing that leaves when iREACHE LASTMILE leaves is iREACHE LASTMILE.

The system stays.

That is what we are building.

That is why we exist.

A Vision Written in Kenema

"A Sierra Leone where every person — in every last-mile community, regardless of distance or poverty — has equitable access to quality community health care delivered by a workforce their own community governs and their own government sustains."

Our Approach

WHO WE ARE

Three-Systems Connection Theory — visually explained

THE ORIGINAL DIAGNOSIS

Sierra Leone's community health crisis is not a CHW performance problem. It is a systems disconnection problem.

Three systems that must govern community health together have been operating in near-total isolation for decades. Every programme that treated CHW performance as the primary problem failed because it addressed a symptom while leaving the disconnection — the disease — untreated.

Why the Standard Diagnosis Is Wrong

The standard diagnosis of Sierra Leone's community health crisis runs as follows: CHWs are poorly trained, inadequately equipped, insufficiently supervised, and uncompensated

— therefore if we address these four deficiencies, health outcomes will improve. This diagnosis is not false. CHWs in Sierra Leone have historically been all of these things. But the diagnosis is incomplete in a way that has made every previous solution temporary.

A CHW who is well trained, well equipped, well supervised, and appropriately compensated but operating inside a disconnected system — where traditional authority structures have no role in health governance, where community surveillance intelligence never reaches DHMT planning, and where the government health system remains institutionally and geographically distant — is a high-performing individual inside a failing system. Individual performance improvements in a disconnected system produce episodic health gains during the programme cycle and systemic collapse when the programme ends. The gains disappear because the system that produced the need for those gains has not changed.

Sierra Leone has three groups of people who each hold something essential for community health.

None of them talk to each other.

That is the problem the REACH 360° Framework was designed to solve. Not CHW performance. Not supply chains. Not financing. Those are real problems. But they are symptoms. The disease underneath all of them is this: three groups of people who each hold something the others need — and who have never been connected into a single governing system.

Let us meet each group.

►System One: The Traditional Authority System — social legitimacy

Who They Are

Walk into any remote village in Kenema district.

You will not find a government official. You will not find a DHMT representative. You may not find a health worker of any kind.

But you will find a chief.

A Section Chief. A Paramount Chief. A Bondo society leader. A women's group. A farmers' cooperative. These are the people who have governed community life in Sierra Leone for generations. Long before the Republic of Sierra Leone existed as a legal entity. Long before any Ministry of Health was established. Long before any donor funded any health programme in any district.

These structures are not ceremonial. They are not advisory. They are governing. They make decisions that communities obey. They settle disputes. They control land. They mobilize community members for collective action. They hold the social trust of

everyone in the chieftaincy in a way that no government official and no NGO programme ever will.

When a paramount chief speaks in a chieftaincy, people listen. Not because they are forced to. Because the chief holds a kind of authority that has been built over generations and earned through the governance of community life.

What They Hold

The Traditional Authority System holds three things that no external programme can replicate or manufacture.

The first is social legitimacy. This is the community's fundamental belief that the chief has the right to govern. It is not given by law. It is not granted by the government. It is held because the community believes it is held. This matters enormously in health care — because community health programmes succeed or fail on the basis of whether communities trust them. A programme endorsed by a paramount chief walks into a community with trust already built. A programme that bypasses the chief walks in as a stranger.

The second is enforcement capacity. If a paramount chief tells a community that ProcCHW visits are a governance expectation — not optional, not a programme activity, but a governance expectation — then households that would otherwise close their doors to a female health worker begin to open them. The chief does not need police. The chief does not need legal authority. The chief has social authority. That is more powerful in a remote chieftaincy than any government mandate.

The third is embedded knowledge. A paramount chief knows their chieftaincy the way no outsider can. They know which families are most vulnerable. They know which households have pregnant women. They know which families cannot afford transport to the health facility. They know the social dynamics — which households trust each other, which do not, which community members other people listen to. This knowledge is invisible to any DHMT planning meeting and any district database. It lives in the chief.

Why Every Programme Ignored This

Every health programme that came to Sierra Leone before iREACHE LASTMILE made the same mistake.

They consulted the chief. They held a community meeting. They asked for the chief's blessing. And then they moved on and ran their programme through government

channels and NGO implementation structures and treated the chief as a stakeholder who had been properly engaged.

They did not give the chief a governing role. They did not give the chief formal decision-making authority over the programme. They did not make the chief accountable for outcomes in their chieftaincy. They did not connect the chief's social enforcement capacity to the health system in any structural way.

This is like having the most trusted person in the room and asking them to stand in the corner while someone else runs the meeting.

The REACH 360° Framework does not make that mistake.

► **System Two: Government Health System — clinical authority**

Who They Are

The Government Health System is what most people think of when they hear the words *health system*.

It is the Ministry of Health in Freetown. The District Health Management Teams in district capitals. The Peripheral Health Units scattered across chieftaincies. The nurses, doctors, midwives, laboratory technicians, and health officers who work inside these structures.

This system was built with international support over decades. It has policies. It has protocols. It has clinical standards. It has legal authority over health resource allocation. It has access to national government budgets and donor financing and the technical assistance of international health organizations.

It is the official health system of Sierra Leone.

What It Holds

The Government Health System holds two things that no other system can access.

The first is **clinical authority**. The government sets the clinical standards that determine what counts as quality care. It certifies health workers. It approves drug protocols. It operates the health facilities where complicated cases go when they are beyond a community health worker's scope. Without clinical authority, a community health programme is operating outside the law. The second is **national resources**. Only the government can put ProcCHW salaries into a national budget line. Only the government can integrate CHW data into the national health information system. Only the government can ensure that community health systems survive donor cycles — because donors come and go, but government budgets, in principle, continue. The path to a sustainable community health system runs through government financing. There is no other path.

The Problem with This System

The Government Health System has a structural weakness that no amount of investment has ever fully solved.

It is far away.

Not just geographically — although that is real. The nearest PHU serving a remote Kenema chieftaincy may be fifteen kilometers away on a road that becomes impassable in the rainy season. That distance kills people. We know it kills people because Aminata walked that distance with a dying newborn and the newborn did not survive.

But the distance is also institutional and social. A DHMT sits in a district town. Its planning meetings happen in offices. Its data comes from facilities. Its understanding of what is happening in a remote chieftaincy is limited to what gets reported through official channels — which, in communities without a functioning community health worker, is almost nothing. The Government Health System plans for populations it cannot fully see. It allocates resources to communities it cannot fully reach. It makes clinical decisions based on data that does not include the most marginalised communities — because the most marginalised communities are exactly the ones least represented in government data systems.

This is not a failure of intent. It is a structural failure of distance. And you cannot solve a distance problem by improving the performance of the institution that is far away. You solve it by connecting that institution to the intelligence that is close.

► System Three: Community Epidemiological System — post-Ebola intelligence

Who They Are

This is the hardest system to see — because it was never formally named until the REACH 360° Framework named it.

It is not an organization. It does not have offices. It does not have staff. It does not appear in any government directory or any donor database.

It is the health knowledge that Sierra Leone's communities hold inside themselves.

Specifically — and this is the part that makes it unique in the world — it is the health knowledge that communities in Eastern Province built by surviving the 2014–2016 Ebola outbreak.

What Happened During Ebola — The Full Story

To understand what communities built, you have to understand what they faced.

In 2014, Ebola entered Sierra Leone. It spread fast. The formal health system was overwhelmed almost immediately. Health facilities were running out of protective equipment. Health workers were getting sick and dying. Government response teams existed but they could not reach every village in every remote chieftaincy fast enough.

In the communities most severely affected — including communities in Kenema district, where iREACHE LASTMILE is headquartered today — people were watching their neighbours die. People were watching family members die. And the formal response was not arriving fast enough to stop it.

So communities did something that no health programme had planned for and no health system had asked them to do.

They started protecting themselves.

How Communities Built Their Own Detection System

Here is exactly what happened — in plain language.

First, community members started watching.

People started paying attention to specific symptoms. Fever. Extreme weakness. Vomiting. Bleeding. They had learned what these symptoms meant. They started noticing when a neighbour showed these signs. They started talking to each other about what they were seeing.

This sounds simple. It is not simple. It is the beginning of community-based disease surveillance — the exact same function that formal epidemiology systems perform, but done by ordinary people with no training and no equipment and no mandate. Just eyes, knowledge, and the urgency that comes from watching people die.

Second, they built alert chains.

When a community member noticed something alarming, they needed a way to tell someone who could respond. They built informal alert pathways. One person tells the Section Chief. The Section Chief tells the Paramount Chief. The Paramount Chief sends word to the nearest health post or response team.

These chains were not written down anywhere. They were not formalised into any protocol. They were improvised under pressure. But they worked. They moved information from the community to the formal response faster than official reporting channels — because the chains ran through trusted relationships, not bureaucratic processes.

Third, they developed isolation understanding through painful experience.

Communities learned — by watching the disease spread from person to person — that contact with a sick person was dangerous. They developed their own approaches to separating sick

people from healthy ones. Not because a government protocol told them to. Because they observed the pattern of transmission with their own eyes and drew the conclusion that anyone observing the same pattern would draw.

The paramount chief and section chief enforced these informal isolation approaches through social authority. This is important. The government had legal authority to enforce isolation. But in remote communities, social authority moves faster and with less resistance than legal authority. A chief saying *this household must be separated* carries more immediate weight in a remote chieftaincy than a government order that takes days to reach the community.

Fourth, they built trust-based reporting.

One of the worst problems during the Ebola response was concealment. Families were hiding sick members because they were terrified of what would happen if they reported them. They had seen houses disinfected, belongings destroyed, family members taken away. The stigma was devastating.

So official reporting channels were failing. People were not calling the hotlines. People were not going to the facilities. The formal surveillance system was receiving incomplete information at exactly the moment it needed complete information most.

Communities in the worst-affected areas developed something that the formal system could not: trusted intermediaries. Specific community members — not government officials, not health workers, but trusted neighbours and community leaders — became the people through whom sick households would communicate. A family would not call the government hotline. But they would tell a trusted community member. And that community member would facilitate the connection to the formal response in a way that felt less threatening.

This was community innovation under emergency conditions. It was not perfect. But it saved lives that the formal system could not have saved without it.

Fifth, they built a collective memory.

By the time Ebola ended in 2016, communities in Eastern Province had eighteen months of lived experience with disease detection and community response. They knew what early symptoms looked like. They knew how fast the disease moved. They knew which contact patterns were risky. They knew how to mobilise community authority for health protection.

This knowledge did not disappear when the outbreak ended.

It stayed.

It lives today in the memory of survivors. In the social agreements that communities made during those months. In the informal networks that were built and tested and proven under the most extreme conditions any community can face.

Why Nobody Recognised It

After Ebola ended, health programmes returned to Eastern Province.

They assessed community health needs. They designed interventions. They trained community health workers in disease surveillance protocols.

But they did not ask communities what they already knew.

Nobody sat down with the survivors of Kenema's Ebola response and said: *You built a disease detection system without any external support. You maintained it under the most difficult conditions imaginable. How did it work? What can we learn from it? How can we formalise it so it continues to function when the next health threat arrives?*

Those questions were never asked.

So the knowledge sat dormant. Informally present in communities. Officially invisible to the health system.

This is what the REACH 360° Framework calls the Community Epidemiological System.

It is not something iREACHE LASTMILE invented. It is something Sierra Leone's communities built — at tremendous cost — and that no health architecture had ever formally recognised and activated until now.

How ProcCHWs connect all three systems simultaneously

ProcCHW as Systems Connector

The repositioning of the Community Health Worker from service delivery agent to systems connector is the theoretical contribution that makes the REACH 360° Framework original. The ProcCHW is the human infrastructure through which the three systems exchange intelligence, align authority, and govern community health together. This is not an additional function layered on top of service delivery. It is the primary function — from which service delivery quality, sustainability, and equity all derive.

She is selected by the traditional authority — so she carries social legitimacy into every household she visits. She reports into the government health system — so her data flows into national DHIS2, her referrals reach government facilities, her work is co-owned by the DHMT. And she activates the community epidemiological intelligence — through the CEIN, through the post-Ebola detection knowledge that her neighbors hold, through the alert networks that communities built during Ebola and that the REACH 360° Framework formalizes as a standing component of Sierra Leone's national surveillance system.

She is not just a health worker delivering services.

She is a connection — made human — between three systems that each hold something essential and that have never, before the REACH 360° Framework, been designed to work together.

REACH 360° Framework architecture diagram

THE REACH 360° FRAMEWORK — ARCHITECTURE AND OVERVIEW

"REACH 360° means complete rotation with no blind spots, full circle connection with self-sustaining governance, and full coverage with no person outside the circle of care. That is not a communications aspiration. It is a programme design specification enforced through the Equity Accountability Standard."

11.1 The Framework Architecture

Level	Name	Content
1	Global and National Foundation	WHO Health System Building Blocks (2007); PHC Alma-Ata 1978 and Astana 2018; UHC Framework; SDGs 1, 2, 3, 5, 10, 13, 17; WHO AI Ethics 2021; Sierra Leone NHSSP; National CHW Policy 2024; Vision 2030 CHW Programme; Free Healthcare Initiative 2010; Sierra Leone IDSR Framework; National eHealth and Digital Health Policy
2	Theoretical Architecture	Three-Systems Connection Theory — Sierra Leone's community health crisis is a systems disconnection problem; Four Constitutional Foundations specific to Sierra Leone's governance reality; Equity as the structural spine of all seven pillars; Connection Quality Index as primary measurement architecture; Intelligent Digital Health Systems Integration as horizontal design dimension
3	Seven REACH 360° Pillars	Connected Health Workforce (CHW) Community Intelligence and Surveillance System (CISS) Last-Mile Supply Chain and Commodity Access (LSCCS) Sustainable Health Financing and Domestic Resource Mobilisation (SHFS) Three-Authority Community Governance and Accountability (TACGS) Proactive Community Health Service Delivery (PCHDS) Driving Sustained Impact Through Strategic Partnerships
4	Continuous Systems Cycle	LISTEN → MAP → CO-CREATE → CONNECT → DELIVER → MEASURE → LEARN → STRENGTHEN → SUSTAIN → SCALE — a closed loop returning to LISTEN at a higher level of system integration with each completed cycle

Level	Name	Content
5	Nine Programme Areas	Maternal and Newborn Health Childhood Disease (iCCM) Immunization and Zero-Dose Reduction Nutrition Systems Sexual and Reproductive Health Community Disease Surveillance Intelligent Digital Health Systems Last-Mile Health Supply Chain Emergency Response and WASH
6	Measurement Architecture	Output Impact (pillar KPIs) → Outcome Impact (health status) → Systems Impact (Connection Quality Index) → IDHS Impact (AI Readiness and Responsible Deployment Index) → Equity Impact (Equity Progress Threshold) → Stronger Community Health Systems Better Outcomes Government Capacity Health Equity Universal Health Coverage

Programme vs System — what makes iREACHE LASTMILE different



The Connection Quality Index explained

THE CONNECTION QUALITY INDEX

"The CQI measures what the REACH 360° Framework most cares about — not what the programme produced, but what changed in the governance systems around the programme because of its presence."

21.1 Why Standard Metrics Are Insufficient

Standard programme monitoring measures outputs. The CQI measures systems connection — whether the three governance systems are genuinely exchanging intelligence, aligning authority, and governing community health together. A programme with high output metrics and a CQI score below 40 is a service delivery programme that will collapse when funding ends. A rising CQI score signals a system being built that will outlast the grant cycle. IDHS supports CQI measurement by automating data collection and aggregation, freeing assessors to focus on qualitative dimensions that require human judgment — which no AI can generate from platform data alone.

CQI Dimension	Indicators (4 per dimension)	Year 3 Target	Year 5 Target	IDHS Support
1. Traditional Authority Governance Integration	% of chiefs describing chiefdom health indicators without prompting; TA governance decisions with measurable programme consequences; stockout events escalated	>70%	>90%	TACGS governance event log analysis; TA Health Governance

CQI Dimension	Indicators (4 per dimension)	Year 3 Target	Year 5 Target	IDHS Support
	through traditional governance; ProcCHW selection legitimacy through TA process			Council meeting record automated scoring
2. Government-Community Intelligence Flow	% DHMT decisions citing community data; CEIN detection-to-DHMT time; % community members stating one health statistic; % dashboard alerts generating DHMT corrective action	>50%	>75%	DHMT dashboard access log analysis; CEIN alert timestamp tracking; automated CQI data extraction from DHIS2
3. Community Health System Navigation Capacity	% referrals without ProcCHW escort; time reduction from danger sign to facility; % maternal deaths reported through community channels; % CHC decisions without DC facilitation	>55%	>70%	Referral completion tracking; transport barrier pattern analysis; CHC governance independence scoring from meeting records

CQI Score	Interpretation	Programme Meaning	Required Action
80–100	Systems deeply connected	REACH 360° fully working — systems will sustain without iREACHE direct presence	Maintain; document as model district; begin iREACHE transition to TA role; initiate IDHS government ownership transfer
60–79	Substantially connected with specific gaps	Framework working but one dimension lagging — targeted intervention required	Targeted intervention in weak CQI dimension; additional DC support; IDHS tool review for that dimension
40–59	Partially connected	Outputs without connections — service delivery trap risk	Programme design review; TACGS renegotiation; IDHS deployment suspended in weak dimensions until human systems established

CQI Score	Interpretation	Programme Meaning	Required Action
Below 40	Insufficiently connected	Structural programme failure — service delivery without systems building	District pause; root cause analysis; Crosswalk divergence alert; all IDHS applications suspended until human systems established; Board-level reporting

Strategy 2030

WHO WE ARE



Five-phase roadmap: Foundation → Architecture → Activation → Scale → Institutionalization

PHASED SYSTEMS ACTIVATION ARCHITECTURE

"The most important implementation lesson in community health systems building: deploy systems before ProcCHWs. Every ProcCHW deployed before governance is established, supply chain is integrated, and IDHS is validated is a ProcCHW deployed into a service delivery role rather than a systems connector role."

Phase	Name	Period	ProcCHWs	Human System Milestones	IDHS Milestones
1	FOUNDATION	Jan–Jun 2026	0 — systems only	All legal registrations; Board constituted; core governance documents; TACGS governance bodies in Kenema pilot chieftaincies; government platform	IDHS literacy integrated into ProcCHW training curriculum. AI tool design within DHIS2 architecture initiated. TRIAD AI Test documented and Board-approved.

Phase	Name	Period	ProcCHWs	Human System Milestones	IDHS Milestones
				configured; equity baseline completed; MoH recognition secured; UNICEF-MCF application submitted	IDHS Readiness Assessment completed. NO AI applications deployed.
2	ARCHITECTURE	Jul–Dec 2026	0 — architecture only	TRIAD Governance Protocols and DHMT MoU signed in Kenema; CHCs constituted; ProcCHW Supervisors trained; all 32 critical-path documents produced; ProcCHW curriculum MoH-accredited; platform fully operational; CEIN activation guide completed	IDHS clinical decision support configured in DHIS2 as program rule extension; PBI anomaly detection rules written; AI alert feedback protocol co-designed with ProcCHW input. NO AI applications deployed.
3	ACTIVATION	Q1–Q2 2027	~250 Kenema	First Kenema ProcCHW cohort trained and deployed. First PBI payment on 1st of month. First CQI assessment. Adaptive management	IDHS iCCM decision support activated in Kenema. PBI anomaly detection active. Basic stockout alerts active. Community feedback AI classification

Phase	Name	Period	ProcCHWs	Human System Milestones	IDHS Milestones
				before expanding to other pilot chieftaincies.	active with CHC contestability. First AI Learning Brief published.
4	SCALE	Q3 2027–2028	800–2,800	Scale to 4 pilot districts after Kenema CQI >60. Architecture phase replicated in each new district before ProcCHW deployment. UNICEF-MCF programme fully activated.	Phase 2 IDHS tools activated after supply chain Optimize complete: 21-day demand forecasting; outbreak detection; zero-dose prediction; personalised supervision. IDHS validated in Kenema before each new district deployment.
5	INSTITUTIONALISATION	2029–2031	6,800	All 16 districts; 70–80% domestic financing; MoH leads all operations; iREACHE transitions to TA role in earliest districts; final evaluation published	Phase 3 IDHS tools active: MUAC trajectory; financing simulation; Krio NLP. Full IDHS government ownership transfer by 2031. iREACHE TA role includes IDHS model governance support for government.